SAMPLE ANSWER · CEREBRAL PALSY · CASE 3

Sample 7-minute ISBAR: Talia

Talia Rossi · 15-year-old girl · School-based occupational therapy, state high school · about 1043 spoken words (≈ 7.5 min at 140 words per minute)

I Introduction

53 words

"Hi, I'm the occupational therapy student who has been reviewing Talia Rossi's exam adjustments this morning. Talia is a 15-year-old Year 10 student with spastic diplegic cerebral palsy, GMFCS level III. I need your help urgently because Talia hit her head yesterday and now has signs that I think need immediate medical assessment."

S Situation

121 words

"Talia told me that yesterday she slipped in a standard toilet cubicle at school and hit her head on the basin, and she didn't tell anyone. She has a swollen, tender area above her left ear. Since last night she's had a worsening headache, she vomited before school, she's sensitive to light, she's yawning and resting her head on the desk, and she was briefly confused about whether her exam was today or tomorrow. She has just stood up and swayed slightly.

"Her teacher aide, Mrs Nguyen, wants to walk her to her English exam, which starts in five minutes. Talia has asked me not to tell her mum. I don't think she should sit the exam or be left alone."

B Background

166 words

"Talia lives with her mum, Gina, and her younger brother. She walks with a posterior walker inside and uses a manual wheelchair for longer distances, with a teacher aide for part of the day. Her physiotherapist's August letter says she's walking safely with no falls this year, and her school file describes her as capable and motivated, with excellent attendance. She moved to this school in Year 7 after a difficult time at her previous school, which helps explain her fear of moving again.

"The accessible toilet near her classrooms is being used to store sports equipment, so she uses a standard cubicle and holds the basin to turn. She also told me she doesn't drink anything at school to avoid the toilet, and her drink bottle is full.

"There's no documentation of the fall, any first aid or anyone checking on her, whether she takes regular medication, or what she's eaten and drunk today. I don't know whether the toilet problem has been raised before."

A Assessment

343 words

"My main concern is that Talia's cluster of signs after a head knock — a headache that's getting worse, vomiting, drowsiness, light sensitivity, a brief episode of confusion and unsteadiness — may indicate a significant head injury, and these are recognised warning signs that need urgent medical assessment today. I can't diagnose that, but it isn't safe to wait.

"Her chattiness and joking are reassuring on the surface, and her physio letter says no falls this year, but that letter predates yesterday's fall. Teenagers often minimise symptoms when they're worried about consequences, which Talia clearly is. Her unsteadiness is also hard to

interpret, because she already has altered balance from her diplegia, so I'd be careful not to put the swaying down to her cerebral palsy.

"Secondly, she has stopped drinking at school to avoid an unsafe toilet. Dehydration might add to her headache and can contribute to constipation and urinary problems, and it shows the environment is restricting a basic self-care occupation. The underlying cause is an access barrier: the accessible toilet is being used for storage, which forced her into a cubicle that isn't safe for her. That's a school responsibility under the Disability Standards for Education, not Talia's fault.

"On confidentiality, I respect that Talia is 15 and has a right to privacy and a say in decisions. But confidentiality has limits when there's a risk to her safety, and with a possible head injury her mum needs to know. I'd be honest with Talia about that rather than make a promise I can't keep. Her fear about changing schools is real and should be taken seriously; fixing toilet access is the best way to address it.

"The exam form, speech-to-text software and camp risk assessment are all important, but none of them is urgent compared with her safety. Sitting an exam now could also worsen her symptoms and wouldn't reflect her ability.

"So my priorities are: first, immediate medical assessment and supervision; second, notifying her mum and the school; third, the toilet access barrier; and the exam paperwork later."

R Recommendation

360 words

"My immediate recommendation is that Talia doesn't go to the exam. I'd ask Mrs Nguyen to stay with her while I come to you, keep her seated and not walking, and watch for any change in how alert she is.

"I'd like your help to activate the school's first aid and medical emergency procedure now. Given the vomiting, worsening headache, drowsiness and confusion, I think an ambulance is warranted rather than waiting for a parent to drive her. If she becomes more drowsy, vomits again or has a seizure, we'd call triple zero straight away.

"I'd like us to contact Gina immediately, ideally with Talia present, so Talia can hear what's being said and have some say in how it's explained. I'd also notify the deputy principal so the incident is documented, both yesterday's fall and today's response.

"I'd ask the deputy principal to defer Talia's exam and apply for special consideration, so she's not disadvantaged.

"For the team, I'd want medical assessment of the head injury, and later, any return-to-learning plan from her doctor, as she may need reduced screen time and cognitive load. Her physiotherapist could review her balance after the fall. Her GP could review her fluid intake and bladder and bowel health.

"For OT, once Talia has been medically cleared, my principal short-term assessment would be an observed toilet transfer and access assessment in the school's toilets with Talia, looking at space, grab rails, flooring and basin height. That would give the school specific, evidence-based recommendations, starting with clearing the accessible toilet. For the longer term, I'd use the COPM with Talia to identify her priorities at school, which I expect will include independence, privacy, keeping her friends and staying at this school.

> "The exam adjustments form would still be completed, but we'd ask for an extension and add rest breaks and screen adjustments if her doctor recommends them. I'd also defer the camp risk assessment to a later meeting.
>
> "I'd follow up with Gina and Talia tomorrow to check how she is, and I'd book a meeting with the school to plan reasonable adjustments for toilet access, so Talia can keep drinking during the day."

Coaching notes *not spoken*

The immediate priority

Talia has red-flag symptoms after a head injury: worsening headache, vomiting, drowsiness, brief confusion and unsteadiness. She needs immediate supervision and urgent medical assessment, and her mum must be told despite her request for secrecy.

Distractors in this case

- **Chatty and joking** — teenagers often mask symptoms when they fear consequences.
- **Physio letter: "no falls this year"** — written before yesterday's fall.
- **Exam form deadline, speech-to-text software, bell for the exam** — none outweighs a possible head injury; deferral and special consideration exist for this situation.
- **Camp risk assessment request** — reasonable, but not today's priority.
- **Request not to tell mum** — confidentiality has limits when safety is at risk; be honest with Talia and involve her in how her mum is told.

Missing information to clarify

- Exact time and mechanism of the fall
- First aid given, if any
- Regular medications
- Food and fluid intake today
- Whether the toilet access issue has been raised before

Useful references

- Disability Standards for Education 2005 (Cth)
- Queensland Paediatric Emergency Care guideline: Head injury – emergency management in children (Children's Health Queensland)

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